INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Avelumab — metastatic Merkel-cell carcinoma

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00535
Version 7

Reference treatment cycle

Treatment stages

01 **Treatment visits**
14-day reference cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

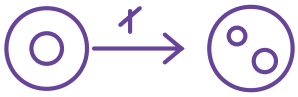
What each medicine does

To help immune cells control metastatic Merkel-cell cancer after chemotherapy; benefit, side effects and suitability for continued treatment are reviewed regularly.

PD-L1 CHECKPOINT IMMUNOTHERAPY

Avelumab

Bavencio



Blocks a PD-L1 immune brake

Avelumab blocks PD-L1, a signal that can reduce the activity of immune cells. It may help the immune system attack Merkel-cell cancer, but can also inflame healthy organs. It is not cytotoxic chemotherapy, even when it follows chemotherapy.

Blocks a PD-L1 immune brake → Treatment plan and safety checks → Report new symptoms early

What to watch for

Tell the nurse immediately about flushing, chills, itching, chest or back pain, dizziness or breathlessness during an infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Confirm the cancer type

Merkel-cell carcinoma is a distinct skin cancer. Keep the pathology report and the sites of metastatic disease; this guide is not for melanoma or urothelial cancer. Ask the team to explain unfamiliar terms in the report.

Where this treatment fits

The current NCCP Merkel-cell indication is metastatic Merkel-cell carcinoma after at least one line of chemotherapy.



Scans and local care

Ask which scans will measure response and whether surgery or radiotherapy has a separate role for symptoms or a particular cancer site. Record their timing in the overall plan.

Your previous treatment

Keep the names, dates, benefits and significant side effects of prior chemotherapy. Bring that history to the immunotherapy discussion rather than assuming the old treatment schedule or toxicity plan still applies.

Your medicines and overall plan

The schedule

Avelumab by intravenous (IV) infusion every 14 days, continuing while beneficial and tolerable. The first four infusions have a specific premedication plan; four is not the course limit.

Important boundaries

This is the Merkel-cell cancer context after chemotherapy under current NCCP. It is not avelumab maintenance for urothelial cancer. Evidence from untreated first-line Merkel-cell cohorts must not be presented as the evidence for this specific post-chemotherapy indication.

Evidence and choices

28 of 88 patients responded (31.8%): eight complete and twenty partial responses; 23 of 28 responses were ongoing at the analysis. These are group results from that study, not a prediction for you. No randomised comparator or demonstrated comparative survival advantage. Trial dose was weight-based; current NCCP uses 800 mg. First-line Part B is a different population and must not replace Part A for this source indication.

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Support alongside treatment

Supportive care addresses symptoms and daily life during cancer treatment. Palliative care can be part of that support; accepting it does not mean you must stop anticancer treatment.

A person you trust

Choose whether to involve a relative or supporter and what may be shared with them. Ask for an interpreter or a different information format. Check that you can explain the next dose, tests and urgent-contact plan in your own words.

Your document set

Keep the current treatment summary, medicine list, test plan and contact card together. Use the printed instructions when you need them; the QR code is an optional link to further information.

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment phases

Avelumab — metastatic Merkel-cell carcinoma

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

14-DAY REFERENCE CYCLE

Treatment visits

Treatment visits **Repeat only as prescribed**

Avelumab by intravenous (IV) infusion every 14 days, continuing while beneficial and tolerable. The first four infusions have a specific premedication plan; four is not the course limit.

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00535 v7: current regimen source

THE WHOLE TREATMENT JOURNEY

Avelumab — metastatic Merkel-cell carcinoma: treatment and checks

Metastatic Merkel-cell carcinoma after chemotherapy

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00535	Blood count, kidney and liver function, glucose and thyroid tests; review prior chemotherapy, immune conditions, previous reactions, transplantation, infection and current medicines. Keep the pathology diagnosis and the agreed metastatic-disease imaging plan.	Blood count, kidney/liver tests, glucose and thyroid tests before every cycle. The nurse watches for infusion reactions and the team checks for bowel, breathing, skin, endocrine and other immune symptoms. Additional tests depend on symptoms, not just the routine blood panel.	Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy. During the infusion, tell the nurse immediately about chills, flushing, itching, chest or back pain, dizziness or breathing difficulty.	Keep reporting possible immune effects after treatment stops. Follow any steroid-taper or hormone-replacement instructions and the individual scan and follow-up plan. There is no automatic four-infusion stop or universal tumour-marker schedule.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00535	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Written plan and contacts NCCP 00535	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1 **Treatment visits**

14-day reference cycle

To help immune cells control metastatic Merkel-cell cancer after chemotherapy; benefit, side effects and suitability for continued treatment are reviewed regularly.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

Avelumab 800 mg is infused over 60 minutes. An antihistamine and paracetamol are given before the first four infusions; NCCP specifies at least 30 minutes beforehand. After that, premedication is at consultant discretion if no reaction occurred. Allow additional time for checks, preparation and observation.

STEP 2

Missed, vomited or held treatment

Contact the unit if a visit is missed or treatment is held. A new appointment and any changed premedication or infusion plan must be confirmed by the team. A severe infusion or immune reaction can change whether treatment is restarted at all.

STEP 3

Appointments and supply

Before each visit, check four things: the blood-test booking, how you will get there, your medicine supply and any help you need. Ask who will review the results and tell you whether the next dose can go ahead.

STEP 4

Paper copies and contacts

Keep the handbook, short guide and treatment card in one place. Ask your team to fill in the real daytime and emergency numbers and print a replacement when the plan changes. A phone is optional.

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Practical arrangements

Write down where blood samples are taken, who receives the results and how you obtain the next appointment or medicine supply. Request a printed plan if that is easier to use.

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

Keep reporting possible immune effects after treatment stops. Follow any steroid-taper or hormone-replacement instructions and the individual scan and follow-up plan. There is no automatic four-infusion stop or universal tumour-marker schedule.

A treatment summary

Ask for a treatment summary showing what you received, important side effects, admissions, the last dose and the follow-up team. Share the current version with your GP and other clinicians.

Your next review

Before leaving, confirm the next test and review, how you will receive results and whom to call sooner. Keep those numbers available after treatment ends.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment visit	 **Day 1** Avelumab 800 mg is infused over 60 minutes. An antihistamine and paracetamol are given before the first four infusions; NCCP specifies at least 30 minutes beforehand. After that, premedication is at consultant discretion if no reaction occurred. Allow additional time for checks, preparation and observation.
Between-visit monitoring	 **Days 2–14** Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy. During the infusion, tell the nurse immediately about chills, flushing, itching, chest or back pain, dizziness or breathing difficulty.

Schedule and review: Avelumab by IV infusion every 14 days, continuing while beneficial and tolerable. The first four infusions have a specific premedication plan; four is not the course limit.

Holds, missed treatment and phase changes: Contact the unit if a visit is missed or treatment is held. A new appointment and any changed premedication or infusion plan must be confirmed by the team. A severe infusion or immune reaction can change whether treatment is restarted at all.

> **Why these bands stay broad:** This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00535 v7: current regimen source ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1: Treatment day

Avelumab 800 mg is infused over 60 minutes. An antihistamine and paracetamol are given before the first four infusions; NCCP specifies at least 30 minutes beforehand. After that, premedication is at consultant discretion if no reaction occurred. Allow additional time for checks, preparation and observation. Report symptoms and previous reactions before treatment.

Day 2–14: Between treatment visits

Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy. During the infusion, tell the nurse immediately about chills, flushing, itching, chest or back pain, dizziness or breathing difficulty. Do not wait for the next appointment to report urgent symptoms.

Support around the medicines

1 Supportive medicines and symptom plan

Avelumab has minimal emetogenic risk in current NCCP. Premedication for the first four infusions prevents or reduces infusion reactions and is distinct from anti-sickness treatment. Confirm the medicine names and timing on the appointment plan. Other symptom treatments depend on the individual; immune-related symptoms need assessment rather than self-treatment alone.

2 Skin and daily care

Report rash and itching early and obtain urgent assessment for blistering, peeling or painful widespread rash. Check your skin and report a new or growing lump or persistent lesion; do not assume every new skin change is a treatment rash.

3 Fertility, pregnancy and breastfeeding

Use effective contraception during treatment and for at least one month after the last avelumab dose. Discuss fertility and breastfeeding with the team before treatment; do not substitute the different interval used for another immunotherapy.

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Blood count, kidney and liver function, glucose and thyroid tests; review prior chemotherapy, immune conditions, previous reactions, transplantation, infection and current medicines. Keep the pathology diagnosis and the agreed metastatic-disease imaging plan.

During treatment

Blood count, kidney/liver tests, glucose and thyroid tests before every cycle. The nurse watches for infusion reactions and the team checks for bowel, breathing, skin, endocrine and other immune symptoms. Additional tests depend on symptoms, not just the routine blood panel.

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

What to expect between visits

A quiet treatment day does not rule out later problems.



Supportive medicines

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

When life is affected

Bring up difficulties with eating, sleep, pain, energy, work or caring for someone. The team can arrange symptom support while cancer treatment continues.

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Food, fluids and activity

Choose food, fluids and movement that you can manage, within any personal restrictions. Ask for help early if you are losing weight, drinking less or finding everyday activity harder.

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sexual health

You can request a private discussion about intimacy, sexual symptoms or relationships. Ask for help if the agreed pregnancy precautions are difficult to follow.

Questions worth asking

Which tests are needed?

Blood count, kidney and liver function, glucose and thyroid tests; review prior chemotherapy, immune conditions, previous reactions, transplantation, infection and current medicines. Keep the pathology diagnosis and the agreed metastatic-disease imaging plan. Blood count, kidney/liver tests, glucose and thyroid tests before every cycle. The nurse watches for infusion reactions and the team checks for bowel, breathing, skin, endocrine and other immune symptoms. Additional tests depend on symptoms, not just the routine blood panel. Keep reporting possible immune effects after treatment stops. Follow any steroid-taper or hormone-replacement instructions and the individual scan and follow-up plan. There is no automatic four-infusion stop or universal tumour-marker schedule.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

The treatment this guide covers This is the Merkel-cell cancer context after chemotherapy under current NCCP. It is not avelumab maintenance for urothelial cancer. Evidence from untreated first-line Merkel-cell cohorts must not be presented as the evidence for this specific post-chemotherapy indication.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Avelumab — metastatic Merkel-cell carcinoma · support plan

Active medicines: Avelumab

Emetogenic risk: Minimal per NCCP 00535 v7

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Avelumab has minimal emetogenic risk in current NCCP. Premedication for the first four infusions prevents or reduces infusion reactions and is distinct from anti-sickness treatment. Confirm the medicine names and timing on the appointment plan. Other symptom treatments depend on the individual; immune-related symptoms need assessment rather than self-treatment alone.
- Blood count, kidney/liver tests, glucose and thyroid tests before every cycle. The nurse watches for infusion reactions and the team checks for bowel, breathing, skin, endocrine and other immune symptoms. Additional tests depend on symptoms, not just the routine blood panel.
- Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy. During the infusion, tell the nurse immediately about chills, flushing, itching, chest or back pain, dizziness or breathing difficulty.
- Use effective contraception during treatment and for at least one month after the last avelumab dose. Discuss fertility and breastfeeding with the team before treatment; do not substitute the different interval used for another immunotherapy.
- This is the Merkel-cell cancer context after chemotherapy under current NCCP. It is not avelumab maintenance for urothelial cancer. Evidence from untreated first-line Merkel-cell cohorts must not be presented as the evidence for this specific post-chemotherapy indication.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / hormones

Immunotherapy

Severe headache, dizziness, profound tiredness, thirst or altered vision can signal pituitary, adrenal, thyroid or glucose problems. Symptoms may need urgent hormone tests and treatment.

3 Heart

Immunotherapy

Rare heart inflammation can cause chest pain, palpitations, faintness or breathlessness. Severe chest pain, collapse or major breathing difficulty is an emergency.

5 Liver

Immunotherapy

Liver inflammation may first appear in blood tests. Yellow eyes or skin, dark urine, severe nausea or right-sided abdominal pain also need prompt review.

7 Nerves / muscles

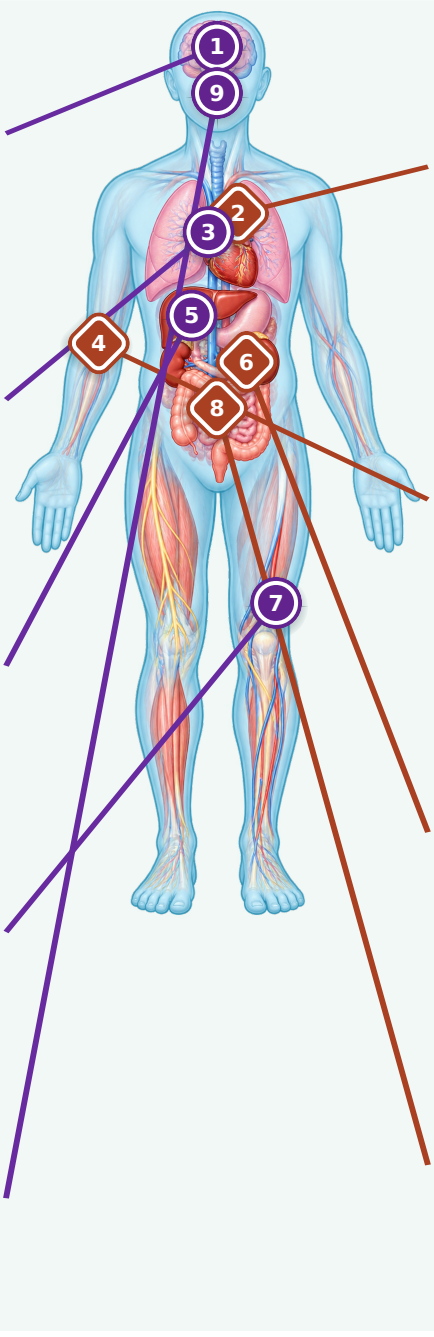
Immunotherapy

New weakness, drooping eyelids, double vision, numbness or difficulty walking, swallowing or breathing may be a rare nerve or muscle effect. Seek urgent help; do not wait for the next visit.

9 Eyes

Immunotherapy

Eye pain, redness, light sensitivity or changed vision can indicate inflammation. Get prompt assessment; do not assume it is ordinary tiredness.



2 Lungs

Immunotherapy

New cough, shortness of breath or chest discomfort can mean lung inflammation or infection. Report changes promptly, even with normal recent blood results.

4 Skin

Immunotherapy

Rash and itching can occur. Rapid spreading, blistering, peeling or sores involving the mouth, eyes or genitals require urgent assessment.

6 Kidneys / urine

Immunotherapy

Kidney inflammation may be detected in blood tests. Report reduced urine, blood in urine, swelling or a marked change in thirst.

8 Bowels

Immunotherapy

New diarrhoea, blood or mucus, severe cramps or abdominal pain can mean bowel inflammation. Record changes from your usual pattern and call early; severe pain or bleeding needs urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00535 v7 supportive-care section; current product information for medicine-specific precautions**
   https://healthservice.hse.ie/documents/8038/535_Avelumab.pdf
2. **Kaufman et al. JAVELIN Merkel 200 Part A, Lancet Oncology 2016**
   https://pubmed.ncbi.nlm.nih.gov/27592805/